



Cataract Surgery with Diabetes

What diabetic patients need to know about cataract surgery, retinopathy screening, and lens selection

Diabetic patients develop cataracts earlier and more aggressively than non-diabetic patients. But the cataract is often the simpler problem. **The real complexity comes from what diabetes does to the rest of the eye**, especially the retina. With the right expertise, excellent outcomes are achievable.

How Diabetes Complicates This

1 Retinopathy Risk

Diabetic retinopathy must be evaluated and often treated before cataract surgery. Surgery can worsen existing retinal disease.

2 Macular Edema

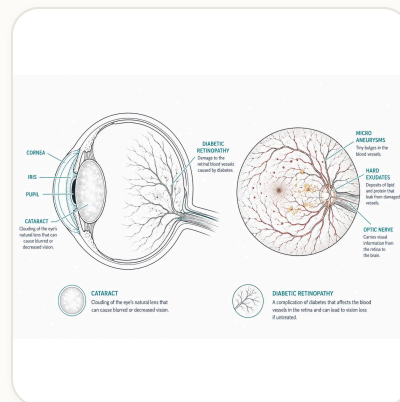
Active swelling in the central retina (DME) limits the visual improvement cataract surgery can deliver and may worsen after the procedure.

3 Slower Healing

Diabetic eyes heal more slowly and are more prone to post-operative inflammation. Blood sugar control directly affects recovery.

4 Lens Selection

Multifocal lenses are generally not ideal for diabetic eyes. Monofocal or EDOF lenses typically provide the most reliable vision.



Diabetic retinopathy and cataract

How I Manage Diabetic Eyes

✓ Full Retinal Evaluation

OCT imaging, retinal photography, and dilated exam before any surgical planning begins.

✓ Retina Treatment In-House

Anti-VEGF injections and laser photocoagulation performed by me, not referred out. One surgeon, one plan.

✓ Staged When Needed

If the retina requires treatment first, we stabilize it before proceeding to cataract surgery. Timing matters.

✓ Appropriate Lens Choice

Lens selection accounts for current retinal status and long-term diabetic trajectory. Conservative choices protect future vision.

✓ Retina Fellowship Training

My retina fellowship training means I manage the complete picture, not just the lens.

What You Should Expect

Diabetic cataract patients may need additional appointments before surgery to evaluate and treat the retina. Recovery can take slightly longer, and close monitoring of blood sugar around surgery improves outcomes. Most diabetic patients achieve significantly improved vision with careful planning.

The Retina Fellowship Advantage

Most cataract surgeons refer diabetic retinopathy to a separate retina specialist. My retina fellowship training means I handle both. Anti-VEGF injections, laser treatment, and cataract surgery, all from one surgeon who understands your complete eye.

Questions to Bring to Your Consultation

- Do I have diabetic retinopathy that needs treatment first?
- Which lens implant is safest given my diabetic status?
- What monitoring will I need after surgery for retinal changes?
- Will you manage my retina and my cataract, or refer me out?
- How should I manage my blood sugar before and after surgery?
- Are there risks if my diabetes is not well controlled right now?

Diabetic and Developing Cataracts?

I evaluate both your cataract and your retina, and build a plan that protects your complete visual system.

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